Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 06-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Study of Efficacy and Safety of Ianalumab Versus Placebo in Addition to Eltrombopag in Primary Immune Thrombocytopenia Patients Who Failed Steroids **Official Title:** A Phase 3 Randomized, Double-blind Study of Ianalumab (VAY736) Versus Placebo in Addition to Eltrombopag in Patients With Primary Immune Thrombocytopenia (ITP) Who Had an Insufficient Response or Relapsed After First Line Steroid Treatment (VAYHIT2) **Short Title/Acronym:** VAYHIT2 **Protocol Number:** CVAY736Q12301 **NCT Number:** NCT05653219 **Posting ID:** 909968739552549681 **Sponsor Name:** Novartis (Novartis Pharma AG) **Investigational Product:** Ianalumab (VAY736) in combination with Eltrombopag (Trade names: Promacta, Alvaiz) **Phase of Development:** Phase III **Trial Status:** Active, Not Recruiting **Medical Monitor:** Novartis Clinical Operations Lead (Contact: Severine Berard)

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effect of two different doses of ianalumab added to eltrombopag to prolong Time to Treatment Failure (TTF) in adults with primary immune thrombocytopenia (ITP) who failed previous first-line treatment with steroids. **Secondary Objectives:** To evaluate the achievement of a stable/sustained platelet response at 6 months, assess Complete Response rate and Response rate at each timepoint, assess safety and tolerability, evaluate reduction in fatigue, facilitate tapering off eltrombopag, and delay the need for subsequent therapy.

2.2 Background & Rationale To address the need for disease-modifying therapies that offer durable responses in Primary ITP. Current second-line treatments, such as thrombopoietin receptor agonists (TPO-RAs), require chronic long-term administration, and a majority of patients relapse after tapering. This study investigates whether the addition of ianalumab (a B-cell depleting monoclonal antibody) to short-term eltrombopag can ameliorate the disease course, prolong the duration of safe platelet levels, and reduce the burden of chronic treatment.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1:1 (Ianalumab 9 mg/kg + eltrombopag : Ianalumab 3 mg/kg + eltrombopag : Placebo + eltrombopag)

3.2 Planned Enrollment & Duration Target N: 152 randomized participants **Number of Sites:** Multicenter (Global) **Estimated Study Start:** December 2022 **Estimated Primary Completion:** August 2025 (Data Readout)

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female patients aged 18 years and older on the day of signing the informed consent. 2. A signed informed consent must be obtained prior to participation in the study. 3. A diagnosis of primary ITP, with insufficient response to, or relapse after a first-line corticosteroid therapy (with or without IVIG). 4. Patient with platelet count < 30 G/L (whom eltrombopag is clinically indicated as per physician's discretion) and with no contraindication to receive eltrombopag.

4.2 Exclusion Criteria 1. ITP patients who received second-line ITP treatments (other than steroid therapy $\pm$ IVIG) including splenectomy. However, patients exposed to thrombopoietin receptor agonists (TPO-RAs) for a limited time (max one week) before screening are eligible. 2. Patients with key lab abnormalities and patients with Evans syndrome or any other cytopenia (patients with low-grade anemia related to bleeding or iron deficiency are eligible). 3. Patients with history of clinically significant hematological disorders, or with marked altered hematologic parameters. 4. Patients with current or history of life-threatening bleeding. 5. Patients that are Human Immunodeficiency Virus (HIV), Hepatitis C Virus (HCV), Hepatitis B surface Antigen (HBsAg)/Hepatitis B core antibody (HBcAb)-positive. HBcAb-positive patients can be enrolled if HBsAg negative, HBV DNA negative, no pre-existing liver fibrosis is present and antiviral prophylaxis is given. 6. Patients with known active or uncontrolled infection requiring systemic treatment during screening period. 7. Patients with hepatic impairment. 8. Patients with concurrent coagulation disorders and/or receiving antiplatelet or anticoagulant medication with an exemption of low dose of acetylsalicylic acid (≤ 150 mg daily). 9. Nursing (breast feeding) or pregnant women.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ianalumab 9 mg/kg or 3 mg/kg (or placebo) administered as four once-monthly infusions. Eltrombopag administered once daily. **Route of Administration:** Intravenous (IV) for ianalumab/placebo; Oral for eltrombopag. **Duration of Treatment:** 24 weeks total (16 weeks of combination therapy followed by an 8-week eltrombopag tapering period).

5.2 Concomitant & Prohibited Therapy Permitted: Treatments for low-grade anemia related to bleeding or iron deficiency. Low dose of acetylsalicylic acid (≤ 150 mg daily). **Prohibited:** Concurrent administration of other second-line ITP treatments or rescue medications unless indicating a treatment failure event.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	2-4 weeks prior	Informed Consent, Medical History, Baseline Labs (Platelet counts)
Baseline	Day 0	Randomization, First Dose of Ianalumab/Placebo and Eltrombopag
Interim	Weeks 1 to 24	Ianalumab infusions (monthly x4), daily eltrombopag monitoring, eltrombopag tapering (weeks 16-24), Safety Labs
End of Study	Up to 39 months	Long-term follow-up for Time to Treatment Failure (TTF), Platelet count stability, Safety tracking

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Time from randomization until treatment failure (TTF). Treatment failure is defined as the time from randomization date until the first of the following events indicative of treatment failure: * Platelet count below 30 G/L * Start of a new ITP treatment * Need for a rescue treatment * Ineligibility to taper or inability to discontinue eltrombopag * Death

7.2 Secondary & Exploratory Endpoints 1. Stable response at 6 months (defined as platelet count $\ge 50 \times 10^9$/L in $\ge 75\%$ of measurements between weeks 19 and 25 without rescue therapy). 2. Complete Response rate at each timepoint. 3. Response rate at each timepoint. 4. Overall frequency of Adverse Events (AEs) and Serious Adverse Events (SAEs).

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via onsite visits and long-term follow-up (up to 39 months). Focus on infection risks given the B-cell depletion mechanism. **Serious Adverse Events (SAEs):** Evaluated continuously (reported at 16% in 9-mg, 6% in 3-mg, and 4% in placebo groups during the study). **Data Monitoring Committee (DMC):** Internal safety monitoring managed by Novartis Pharma AG to assess ongoing benefit-risk profiles.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy (TTF). Safety Set for all AE/SAE evaluations. **Sample Size**

Justification: 152 patients randomly assigned 1:1:1 to ensure appropriate power ($\alpha = 0.05$) to detect statistically significant improvements in TTF for ianalumab + eltrombopag vs. placebo + eltrombopag. **Handling of Missing Data:** Use of rescue medication or start of a new ITP therapy is treated directly as a "Treatment Failure" event in the TTF survival analysis.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits for data source verification across global centers. **Audit Trail:** Validated eCRF locking, tracking of laboratory blinding protocols, and data clarification mechanisms.

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11. Study Identifiers & Governance

Primary ID: CVAY736Q12301 **Registry Number:** NCT05653219 **Posting ID:** 909968739552549681 **Sponsor/Lead Organization:** Novartis **Trial Status:** ACTIVE_NOT_RECRUITING **Study Title:** A Study of Efficacy and Safety of Ianalumab Versus Placebo in Addition to Eltrombopag in Primary Immune Thrombocytopenia Patients Who Failed Steroids **Official Regulatory Title:** A Phase 3 Randomized, Double-blind Study of Ianalumab (VAY736) Versus Placebo in Addition to Eltrombopag in Patients With Primary Immune Thrombocytopenia (ITP) Who Had an Insufficient Response or Relapsed After First Line Steroid Treatment (VAYHIT2)

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12. Clinical Framework (ITP Specific Adaptation)

Primary Condition: Primary Immune Thrombocytopenia (ITP) **Diagnosis Threshold:** Platelet count < 30 G/L (30×10^9/L) **Medical Methodology:** B-cell activating factor (BAFF) receptor blockade (ianalumab) combined with TPO-RA (eltrombopag) **Optimization Target:** Sustained platelet count $\ge 50 \times 10^9$/L without need for rescue treatment

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13. Study Procedures & Methodology

Management Pathway: Step-up second-line ITP combination therapy followed by TPO-RA tapering **Education Protocol (HCP):** IV infusion administration guidelines, TPO-RA tapering schedule, and bleeding risk monitoring **Education Protocol (Patient):** Bleeding symptom awareness, adherence to daily oral eltrombopag, onsite visit expectations **Quality Audit Mechanism:** Laboratory standardization for platelet counts and strict tracking of rescue therapy initiation

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14. Observation & Follow-Up Schedule

Total Duration: Up to 39 months (follow-up period) **Onsite Visit Cadence:** Regular monitoring during the 24-week treatment and tapering period, followed by periodic visits **Remote Monitoring Frequency:** Based on clinical site protocol and follow-up phase **Checklist Review Interval:** Routine tracking of bleeding scores and new medications

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	**[DD-MMM-YYYY]**
Clinical Operations Lead	**[Name]**	___	***[DD-MMM-YYYY]***
Lead Statistician	***[Name]***	_____	[DD-MMM-YYYY]